

The Global Burden of Mental Illness

Prevalence, Impact, and the Treatment Gap

Global data overview | 2023

Source: Our World in Data — Mental Health (IHME, Global Burden of Disease 2025)



Hundreds of Millions Affected: 1 in 3 Women and 1 in 5 Men Face Major Depression

1 in 3

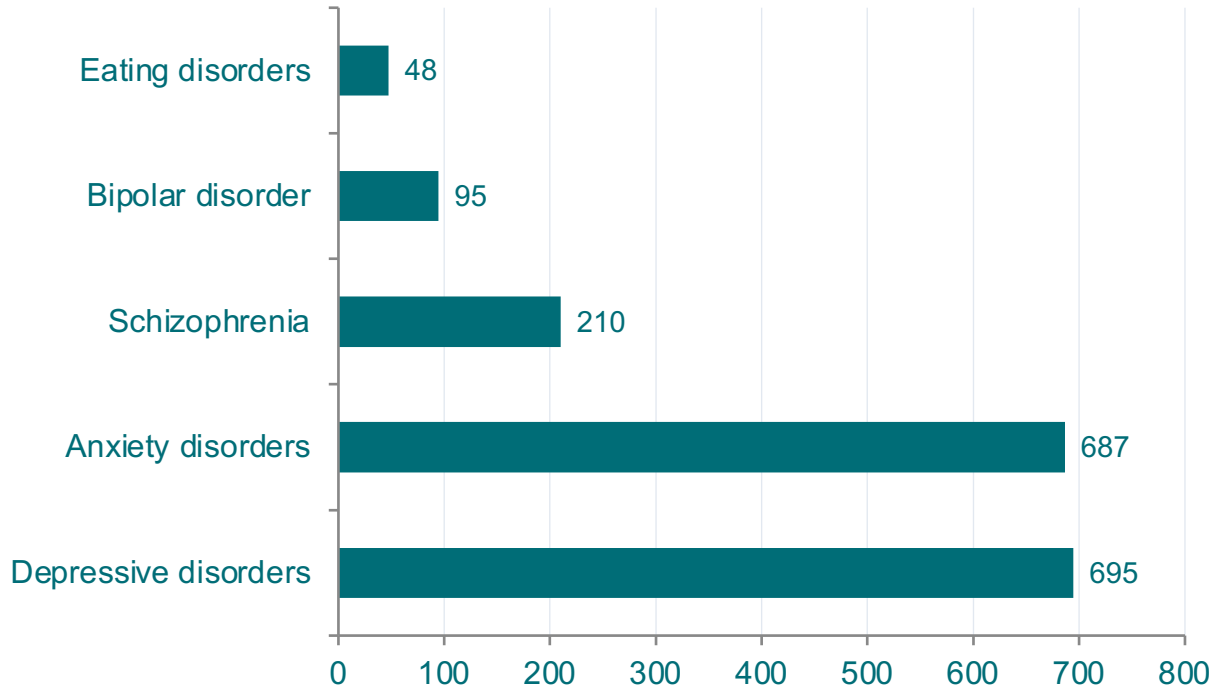
women will experience
major depression in their lifetime

1 in 5

men will experience
major depression in their lifetime

Mental health conditions span from common disorders (depression, anxiety) to less common but severe conditions (schizophrenia, bipolar). Hundreds of millions of people are affected each year, with profound consequences for well-being, work, and relationships.

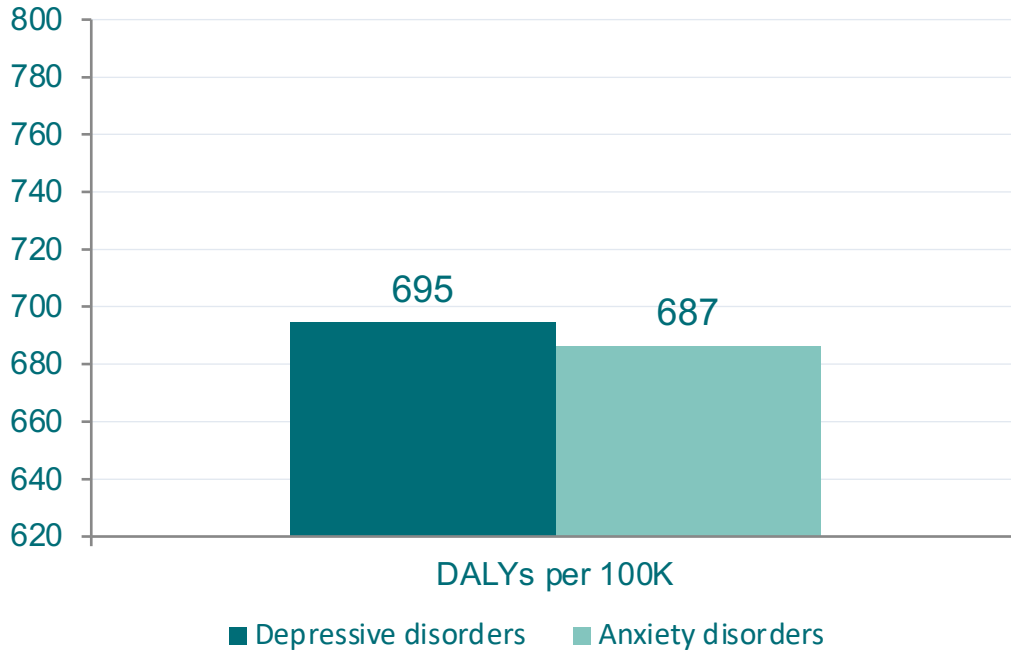
Depression and Anxiety Dominate: ~695 and ~687 DALYs per 100K Respectively



Key Insight

Depressive and anxiety disorders together account for roughly 80% of the total mental health disease burden across these five categories.

Depression and Anxiety Carry Nearly Equal Disease Burden Worldwide



694.6

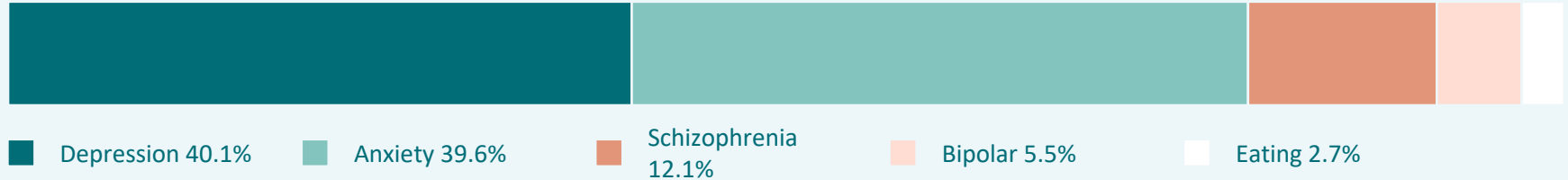
Depressive disorders
DALYs per 100,000

686.5

Anxiety disorders
DALYs per 100,000

Together they dwarf schizophrenia, bipolar, and eating disorders combined — yet both receive far less policy attention than their burden warrants.

Schizophrenia and Bipolar Disorder: Lower Prevalence but Severe Individual Impact



210.2

Schizophrenia DALYs per 100K

While affecting fewer people, schizophrenia reflects severe disability and high individual burden.

94.6

Bipolar disorder DALYs per 100K

A smaller population-level share, yet each case carries substantial disability and care needs.

Treatment Is Available but Often Lacking or Poor Quality



Stigma

Many people are uncomfortable disclosing symptoms to healthcare professionals or even to people they know.



Access

Treatment is often lacking, especially in low- and middle-income countries where resources are scarce.



Quality

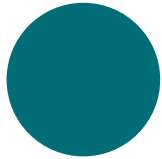
Even where treatment exists, it may be of poor quality — insufficient training, outdated protocols, or underfunding.



Underreporting

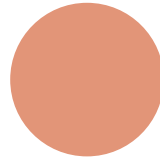
Discomfort with disclosure makes it difficult to estimate true prevalence; the actual burden may be higher than reported.

How People Cope: From Medication to Faith to Family Conversations



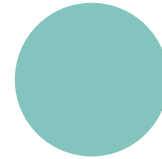
Prescribed Medication

Common in higher-income settings where pharmacological treatment is more accessible and culturally accepted.



Religious / Spiritual Activities

Prevalent in many regions worldwide; faith-based coping is a major pathway for managing anxiety and depression.



Talking to Friends or Family

A widely reported coping mechanism across cultures, underscoring the protective role of social support networks.

The diversity of coping strategies underscores that mental health responses are shaped by cultural, economic, and healthcare system factors. Policy must account for these differences.

Key Takeaways

- 1 Depression and anxiety together represent the overwhelming majority of mental health DALYs.
- 2 Schizophrenia and bipolar carry disproportionate individual burden relative to prevalence.
- 3 Treatment exists but access and quality remain inadequate across much of the world.
- 4 Stigma suppresses both help-seeking and accurate data, masking the true scale of need.
- 5 Policy should scale evidence-based interventions for common disorders while maintaining specialized care for severe conditions.